

For the use only of a registered Medical Practitioner or a Hospital or a Laboratory

DESTACURE
(Desloratadine Syrup 2.5mg/5ml)

COMPOSITION:
Each 5ml contains:
Desloratadine 2.5mg

DESCRIPTION:
Destacure is orange coloured syrupy liquid. It contains Desloratadine as active ingredient and Sucrose, Glycerol, Propylene Glycol, Disodium Edetate, Citric Acid Monohydrate, Color Sunset Yellow Supra, Sodium Benzoate, Sodium Citrate, Ess. Mix Fruit and Purified Water as excipients.

PHARMACODYNAMICS /PHARMACOKINETICS:

Pharmacodynamic Properties:
Pharmacotherapeutic group: antihistamines – H1 antagonist,
ATC code: R06A X27

Mechanism of action
Desloratadine is a non-sedating, long-acting histamine antagonist with selective peripheral H1-receptor antagonist activity. After oral administration, desloratadine selectively blocks peripheral histamine H1-receptors because the substance is excluded from entry to the central nervous system.

Desloratadine has demonstrated antiallergic properties from *in vitro* studies. These include inhibiting the release of proinflammatory cytokines such as IL-4, IL-6, IL-8, and IL-13 from human mast cells/basophils, as well as inhibition of the expression of the adhesion molecule P-selectin on endothelial cells. The clinical relevance of these observations remains to be confirmed.

Pharmacokinetic properties:

Absorption:
From already published literature it is concluded that desloratadine plasma concentrations can be detected within 30 minutes of desloratadine administration in adults and adolescents. Desloratadine is well absorbed with maximum concentration achieved after approximately 3 hours; the terminal phase half-life is approximately 27 hours. The degree of accumulation of desloratadine was consistent with its half-life (approximately 27 hours) and a once daily dosing frequency. The bioavailability of desloratadine was dose proportional over the range of 5 mg to 20 mg.

Distribution:
Desloratadine is moderately bound (83 % - 87 %) to plasma proteins. There is no evidence of clinically relevant active substance accumulation following once daily adult and adolescent dosing of desloratadine (5 mg to 20 mg) for 14 days.

Biotransformation:
The enzyme responsible for the metabolism of desloratadine has not been identified yet, and therefore, some interactions with other medicinal products cannot be fully excluded. Desloratadine does not inhibit CYP3A4 *in vivo*, and *in vitro* studies have shown that the medicinal product does not inhibit CYP2D6 and is neither a substrate nor an inhibitor of P-glycoprotein.

Elimination:
In a single dose of desloratadine, there is no effect of food (high-fat, high caloric breakfast) on the disposition of desloratadine.

INDICATIONS:
DESTACURE is indicated for the rapid relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge and itching, congestion/stuffiness, as well as ocular itching, tearing and redness.
DESTACURE is also indicated for the relief of symptoms associated with urticaria such as the relief of itching and the size and number of hives.

DOSE AND ADMINISTRATION:
The prescriber should be aware that most cases of rhinitis below 2 years of age are of infections origin and there are no data supporting the treatment of infectious rhinitis with DESTACURE.

Children 6 through 11 years of age: 5 ml (2.5 mg) DESTACURE once a day, with or without a meal for the relief of symptoms associated with allergic rhinitis (including intermittent and persistent allergic rhinitis) and urticaria.

Children 1 through 5 years of age: 2.5 ml (1.25 mg) DESTACURE once a day, with or without a meal for the relief of symptoms associated with allergic rhinitis (including intermittent and persistent allergic rhinitis) and urticaria.

In adults and adolescents (12 years of age and over): 10 ml (5 mg) DESTACURE once a day, with or without a meal for the relief of symptoms associated with allergic rhinitis (including intermittent and persistent allergic rhinitis) and urticaria.

Intermittent allergic rhinitis (presence of symptoms for less than 4 days per week or less than 4 weeks) should be managed in accordance with the evaluation of patient's disease history and the treatment could be discontinued after symptoms are resolved and reinitiated upon their reappearance. In persistent allergic rhinitis (presence of symptoms for 4 days or more per week and for more than 4 weeks), continued treatment may be proposed to the patients during allergen exposure periods.

CONTRAINDICATIONS:
Hypersensitivity to the active substance, to any of the excipients used in the formulation.

WARNINGS AND SPECIAL PRECAUTIONS FOR USE:
Desloratadine should be administered with caution in patients with medical or familial history of seizures, and mainly young children, being more susceptible to develop new seizures under Desloratadine treatment. Healthcare providers may consider discontinuing Desloratadine in patients who experience a seizures while on treatment.

Paediatric population
In children below 2 years of age, the diagnosis of allergic rhinitis is particularly difficult to distinguish from other forms of rhinitis. The absence of upper respiratory tract infection or structural abnormalities, as well as patient history, physical examinations, and appropriate laboratory and skin tests should be considered.

Approximately 6 % of adults and children 2- to 11-year old are phenotypic poor metabolisers of desloratadine and exhibit a higher exposure. The safety of desloratadine in children 2- to 11-years of age who are poor metabolisers is the same as in children who are normal metabolisers. The effects of desloratadine in poor metabolisers < 2 years of age have not been studied.

In the case of severe renal insufficiency, DESTACURE should be used with caution.
This medicinal product contains sucrose; thus, patients with rare hereditary problems of fructose intolerance, glucose-galactosemalabsorption or sucrase-isomaltase insufficiency should not take this medicine.

INTERACTIONS WITH OTHER MEDICAMENTS:
From published literature of innovator it is found that no clinically relevant interactions was observed in clinical trials with desloratadine in which erythromycin or ketoconazole were co-administered.

Paediatric population
Interaction studies of innovator have only been performed in adults by innovator.
In a published clinical pharmacology trial, desloratadine tablets taken concomitantly with alcohol did not potentiate the performance impairing effects of alcohol. However, cases of alcohol intolerance and intoxication have been reported

during post-marketing use. Therefore, caution is recommended if alcohol is taken concomitantly.

PREGNANCY AND LACTATION:

Pregnancy
Published data shows that a large amount of data on pregnant women (more than 1,000 pregnancy outcomes) indicate no malformative nor foeto/ neonatal toxicity of desloratadine. Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. As a precautionary measure, it is preferable to avoid the use of DESTACURE during pregnancy.

Breast-feeding
Desloratadine has been identified in breastfed newborns/infants of treated women. The effect of desloratadine on newborns/infants is unknown. A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from DESTACURE therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

Fertility
There are no data available on male and female fertility.

UNDESIRABLE EFFECTS:

Summary of the safety profile		
System Organ Class	Frequency	Adverse reactions
Metabolism & nutrition disorders	Not known	Increased appetite
Psychiatric disorders	Very rare Not known	Hallucinations Abnormal behavior, aggression
Nervous system disorders	Common Common (children less than 2 years) Very rare	Headache Insomnia Dizziness, somnolence, insomnia, psychomotor hyperactivity, seizures
Cardiac disorders	Very rare Not known	Tachycardia, palpitations QT prolongation
Gastrointestinal disorders	Common Common (children less than 2 years) Very rare	Dry mouth Diarrhoea Abdominal pain, nausea, vomiting, dyspepsia, diarrhoea
Hepatobiliary disorders	Very rare Not known	Elevations of liver enzymes, increased bilirubin, hepatitis Jaundice
Skin & subcutaneous tissue disorders	Not known	Photosensitivity
Musculoskeletal & connective tissue disorders	Very rare	Myalgia
General disorders & administration site conditions	Common Common (children less than 2 years) Very rare Not known	Fatigue Fever Hypersensitivity reactions (such as anaphylaxis, angioedema, dyspnoea, pruritus, rash, and urticaria) Asthenia
Investigations	Not known	Weight increased

Paediatric population
Other undesirable effects reported from published literature included QT prolongation, arrhythmia, bradycardia, abnormal behavior and aggression.

OVERDOSE AND TREATMENT:
The adverse event profile associated with overdosage, is similar to that seen with therapeutic doses, but the magnitude of the effects can be higher.

Treatment
In the event of overdose, consider standard measures to remove unabsorbed active substance.
Symptomatic and supportive treatment is recommended.
Desloratadine is not eliminated by haemodialysis; it is not known if it is eliminated by peritoneal dialysis.

Symptoms
No clinically relevant effects are observed.

Paediatric population
The adverse event profile associated with overdosage, is similar to that seen with therapeutic doses, but the magnitude of the effects can be higher.

EFFECT ON ABILITY TO DRIVE AND USE MACHINES
Destacure has no or negligible influence on the ability to drive and use machines based on published clinical trials. Patients should be informed that most people do not experience drowsiness. Nevertheless, as there is individual variation in response to all medicinal products, it is recommended that patients are advised not to engage in activities requiring mental alertness, such as driving a car or using machines, until they have established their own response to the medicinal product.

DOSAGE FORM AND PACKAGING AVAILABLE:
Dosage Form : Syrup
Packing : 60 ml syrup packed in Amber coloured PET bottle.

SHELF LIFE

36 months
STORAGE: Store below 30°C. Do Not Freeze.
Keep the medicine out of reach of children.

Manufactured in India by:
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HEALOL PHARMACEUTICALS SDN. BHD

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